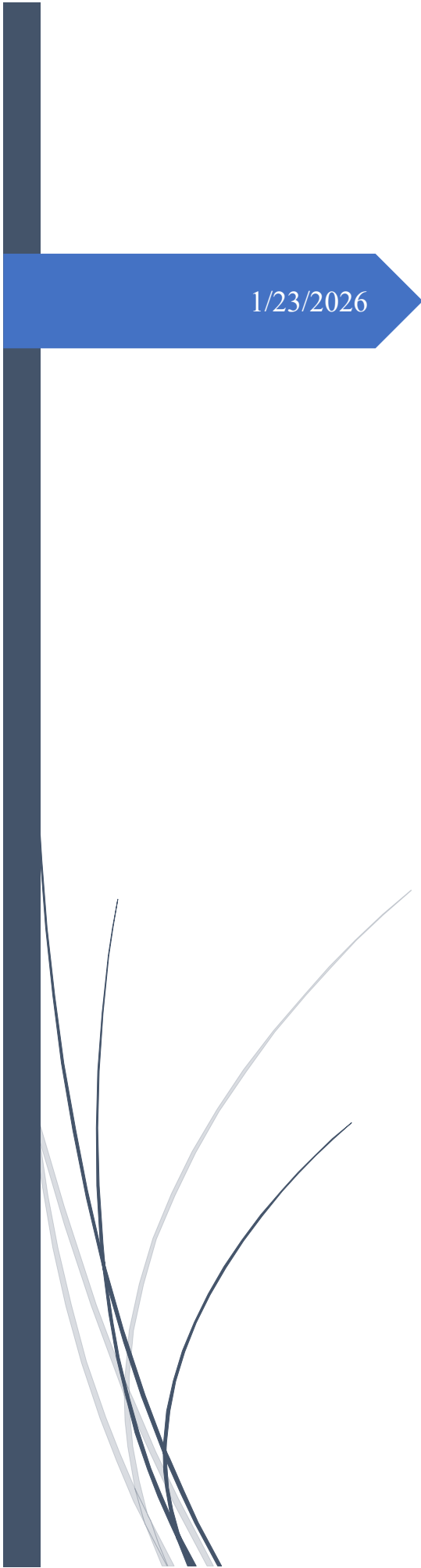




1/23/2026

HIV Treatment Cascade Report

December 2025

Strategic Information Component
HIV/STI/TB UNIT

Table of Contents

Introduction.....	1
National 90-90-90 Treatment Cascade.....	2
RHAs Linked to Care Cascade	4
North East Regional Health Authority (NERHA).....	4
South East Regional Health Authority (SERHA)	5
Southern Regional Health Authority (SRHA).....	6
Western Regional Health Authority (WRHA)	7
Non-RHA Organizations Linked to Care Cascade.....	8
Jamaica AIDS Support for Life (JASL)	8
CHARES	9
AIDS Healthcare Foundation (AHF)	10
Health Connect Jamaica.....	11
UHWI – Paediatric Clinic	12
Bellevue Hospital	13
Department of Correctional Facilities.....	14
RHA Service Level Agreement Targets and Performance	15
Addressing the Gaps	18
Lost to Follow Up	19
PLHIV Not Retained on ARV	21
PLHIV with No Valid Viral Load Test Result.....	23
Plan of Action to Address Data Quality Gaps.....	25
Plan of Action to Address Programmatic Gaps.....	26
Conclusion	26

List of Tables

Table 1: PLHIV Retained on ARV as of December 2025 matched against the Annual Target.....	15
Table 2: PLHIV Viral Load Testing of those eligible for a viral load test as of December 2025 matched against the Annual Target.....	16
Table 3: PLHIV Viral Suppression of those eligible for a viral load test as of December 2025 matched against the Annual Target.....	17

List of Figures

Figure 1: National 90-90-90 Cascade 2024/2025	2
Figure 2: National 90-90-90 Cascade – Quarterly	3
Figure 3: NERHA Linked to Care Cascade – Quarterly	4
Figure 4: SERHA Linked to Care Cascade – Quarterly.....	5
Figure 5: SRHA Linked to Care Cascade – Quarterly	6
Figure 6: WRHA Linked to Care Cascade – Quarterly.....	7
Figure 7: Jamaica AIDS Support for Life (JASL) Linked to Care Cascade – Quarterly.....	8
Figure 8: CHARES Linked to Care Cascade – Quarterly.....	9
Figure 9: AIDS Healthcare Foundation (AHF) Linked to Care Cascade – Quarterly	10
Figure 10: Health Connect Jamaica Linked to Care Cascade – Quarterly.....	11
Figure 11: UHWI – Paediatric Clinic Linked to Care Cascade – Quarterly.....	12
Figure 12: Bellevue Hospital Linked to Care Cascade – Quarterly.....	13
Figure 13: Department of Correctional Facilities Linked to Care Cascade – Quarterly.....	14
Figure 14: HIV Treatment Cascade - Quarterly	18
Figure 15: PLHIV Lost to Follow Up disaggregated by years lost – Quarterly	19
Figure 16: LTFU/LTC disaggregated by RHAs/non-RHAs – Quarterly	20
Figure 17: PLHIV Not Retained on ARV/RIC disaggregated by RHAs/non-RHAs - Quarterly	21
Figure 18: PLHIV retained in care but not retained on ARV disaggregated by days since last clinic visit – Quarterly	22
Figure 19: PLHIV with No Viral Load Test/ On ARV disaggregated by RHAs/non-RHAs - Quarterly	23
Figure 20: Number of PLHIV with no valid viral load test disaggregated by eligibility	24

Introduction

Jamaica has both a concentrated (prevalence $>5\%$ in at least one key population) and a generalized (prevalence rate of 1.1% in the general population) epidemic. The incidence: prevalence ratio (IPR) for epidemic control should be <0.03 . A downward trend in the number of new infections and a reduction in deaths/increasing survival among persons with HIV are required to achieve epidemic control. At the end of 2024, the IPR was estimated at 4.46 compared to 4.62 in 2023. There were an estimated 1,184 new infections for the year, and the number of deaths from all causes among PLHIV was calculated to be 907 (UNAIDS, 2024).

The national treatment cascade comprehensively represents the country's achievement of the UNAIDS target for epidemic control. The aim was for 90% of PLHIVs to know their status, 90% of PLHIVs to know their status to be retained on ARVs, and 90% of PLHIVs retained on ARVs to be virally suppressed by the end of 2025.

This report compares the national HIV treatment cascades for March 2025, June 2025, September 2025 and December 2025, and explores the treatment cascades and lost to follow-up data for the regional health authorities (RHAs) and Non-RHAs. The Service Level Agreement (SLA) targets are included in the report for the RHAs.

A catastrophic category five Hurricane struck the island in October 2025 causing damage to health facilities, power outage and interruption in internet connectivity. This affected real time data entry and access to the Treatment Site Information System (TSIS) especially among the most affected parishes in the WRHA and SRHA. Displacement of health centre workers and patients in the affected parishes further compounded the routine updating of TSIS and health seeking behaviours of PLHIV, which may have impacted the treatment cascade during the period of October to December 2025.

National 90-90-90 Treatment Cascade

The estimated number of persons living with HIV (PLHIV) increased from 26,000 to 28,000 as of January 2025, based on the UNAIDS Spectrum process produced using models and software tools and was informed by HIV surveillance data, surveys, and programmatic data, producing historical trends and projections of critical indicators (Figure 1 & 2).

As of January 2025, the number of PLHIV diagnosed increased from 23,500 to 26,500 based on data obtained from the National Surveillance Unit, which increased the percentage of PLHIV diagnosed from 90% in December 2024 to 95% in March 2025 (Figure 1 & 2). As a result of this increase in the diagnosed pillar, retained on ARVs declined.

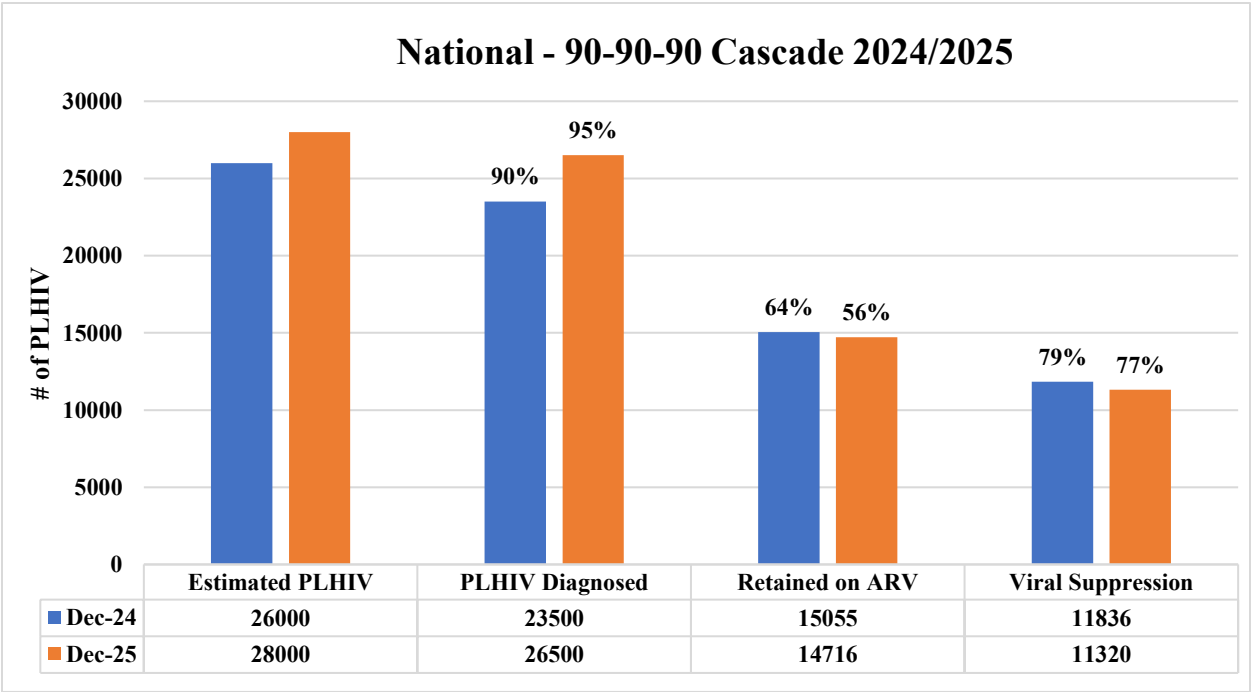


Figure 1: National 90-90-90 Cascade 2024/2025

Comparing the retained in ARV and viral suppression data from December 2024 to December 2025, retention on ARV declined by 8% and viral suppression declined by 2% (Figure 1).

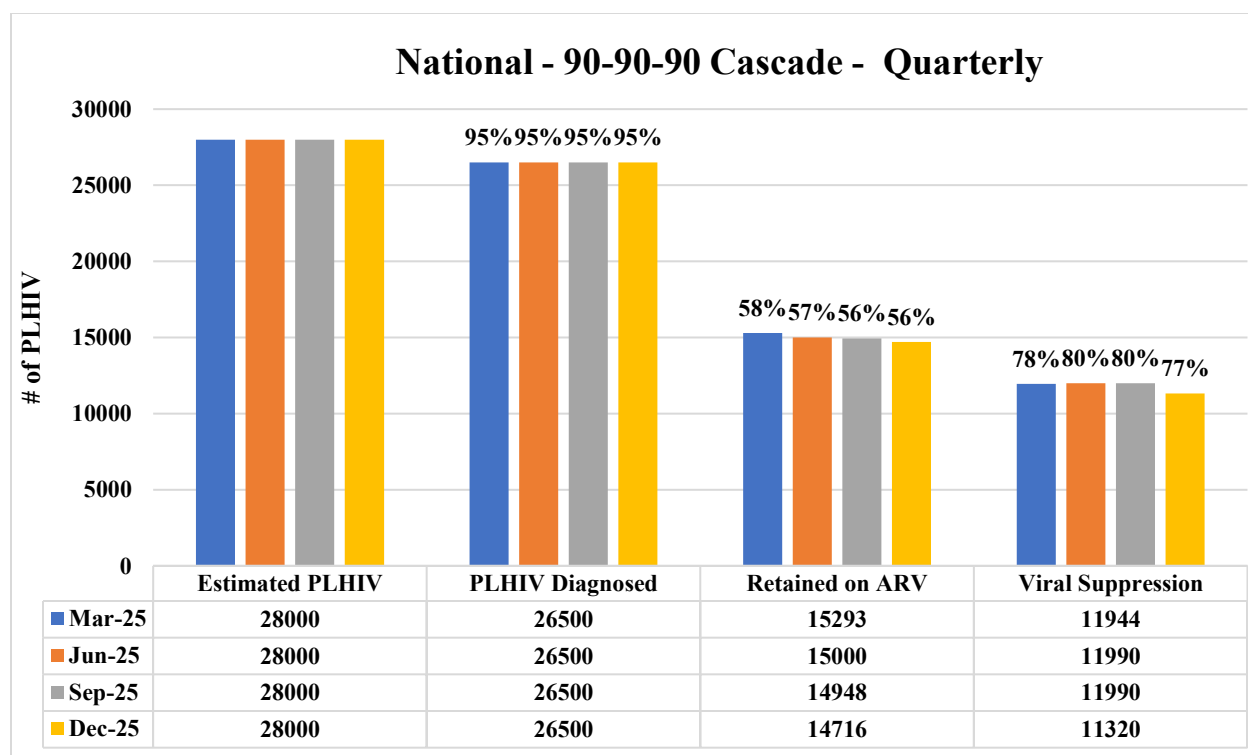


Figure 2: National 90-90-90 Cascade – Quarterly

Comparing data from March 2025 to December 2025, for the National HIV Cascade, with a focus on the retained on ARV and viral suppression pillars, retained on ARV declined by 2% and viral suppression declined by 1% (Figure 2).

Though gains have been made in diagnosing persons living with HIV, these are not translating into sustained retention on ARV and viral suppression.

RHAs Linked to Care Cascade

The RHAs 90-90-90 cascades assess the percentage achievement of the UNAIDS target to achieve epidemic control of HIV. This analysis helps determine how service providers manage the patients linked to care, retained on ARVs, and virally suppressed. The gaps at each pillar of the cascade were identified and can be used to plan targeted interventions to address the fallout at each pillar.

North East Regional Health Authority (NERHA)

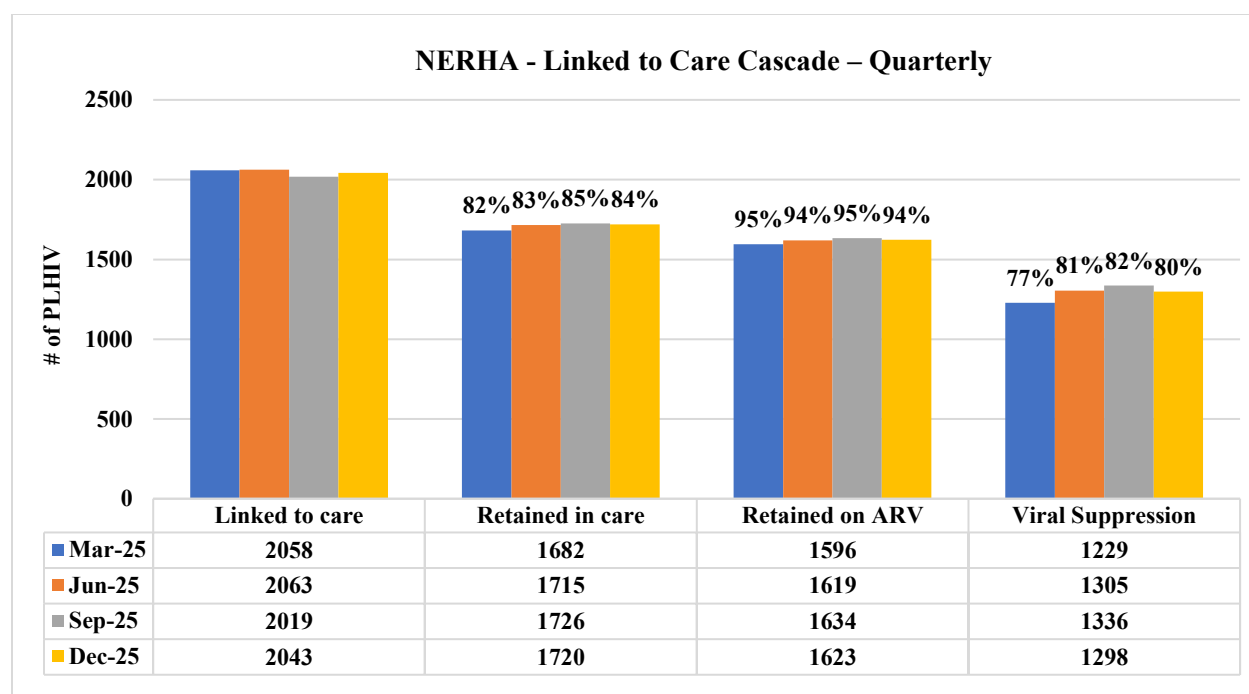


Figure 3: NERHA Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, NERHA's number of clients linked to care declined by 15, due to data-cleaning activities, which included transferring patients and archiving those who had migrated, died, or met the case criteria. Among those linked to care, retention in care increased by 2%, from 82% in March 2025, when compared to 84% in December 2025. This improvement can be attributed to PLHIV returning to care, adherence to data-cleaning standard operating procedures that improved the denominator, and enhanced timeliness of data entry.

Among those retained in care and on ARVs, the percentage declined by 1%, from 95% in March 2025, when compared to 94% in December 2025. This decline can be attributed to data entry error, missed appointments due to Hurricane Melissa and timeliness of data entry. The percentage of PLHIV who were virally suppressed increased by 3%, from 77% in March 2025, when compared to 80% in December 2025, which can be attributed to increased viral load testing and more timely processing and entry of results into the database (Figure 3).

South East Regional Health Authority (SERHA)

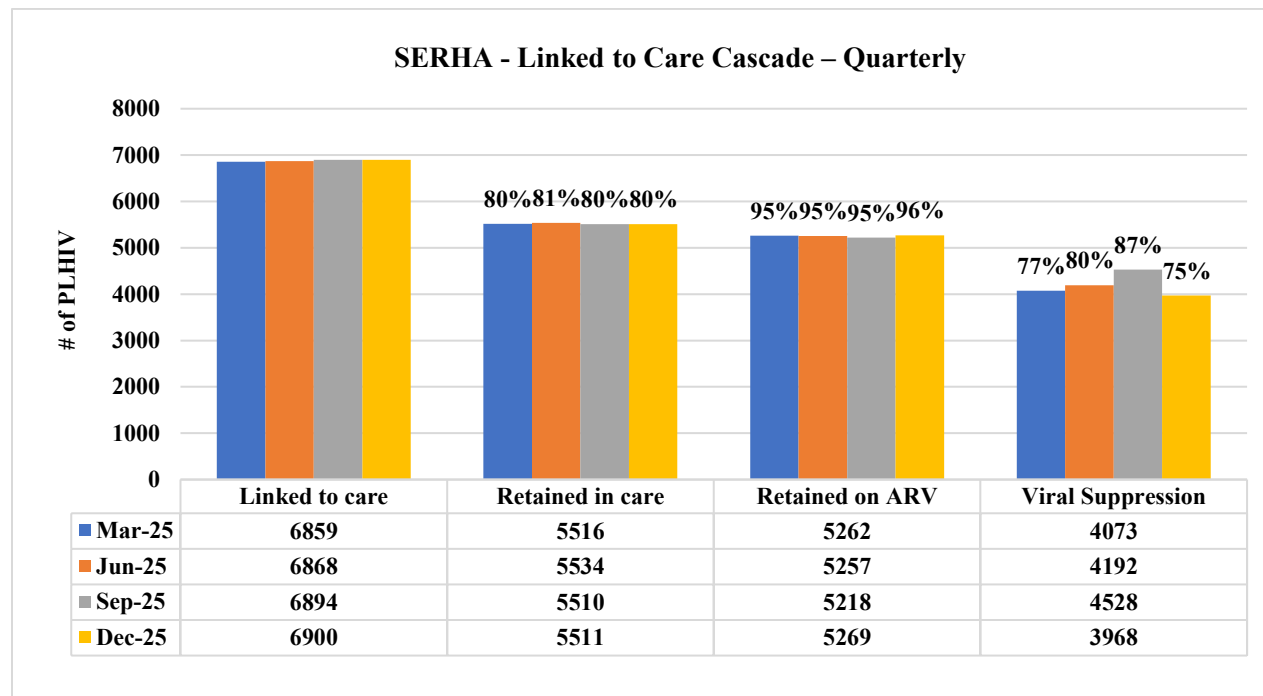


Figure 4: SERHA Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, SERHA's number of clients linked to care increased by 41. Among those linked to care, retention in care remained the same at 80% from March 2025, when compared to December 2025.

Among those retained in care and on ARVs, the percentage increased by 1% from 95% in March 2025 to 96% in December 2025. The percentage of PLHIV who were virally suppressed declined by 2%, from 77% in March 2025, when compared to 75% in December 2025, which can be attributed to the timeliness of data entry, the delay of viral load test results and patients refusing to get their viral load test done (Figure 4).

Southern Regional Health Authority (SRHA)

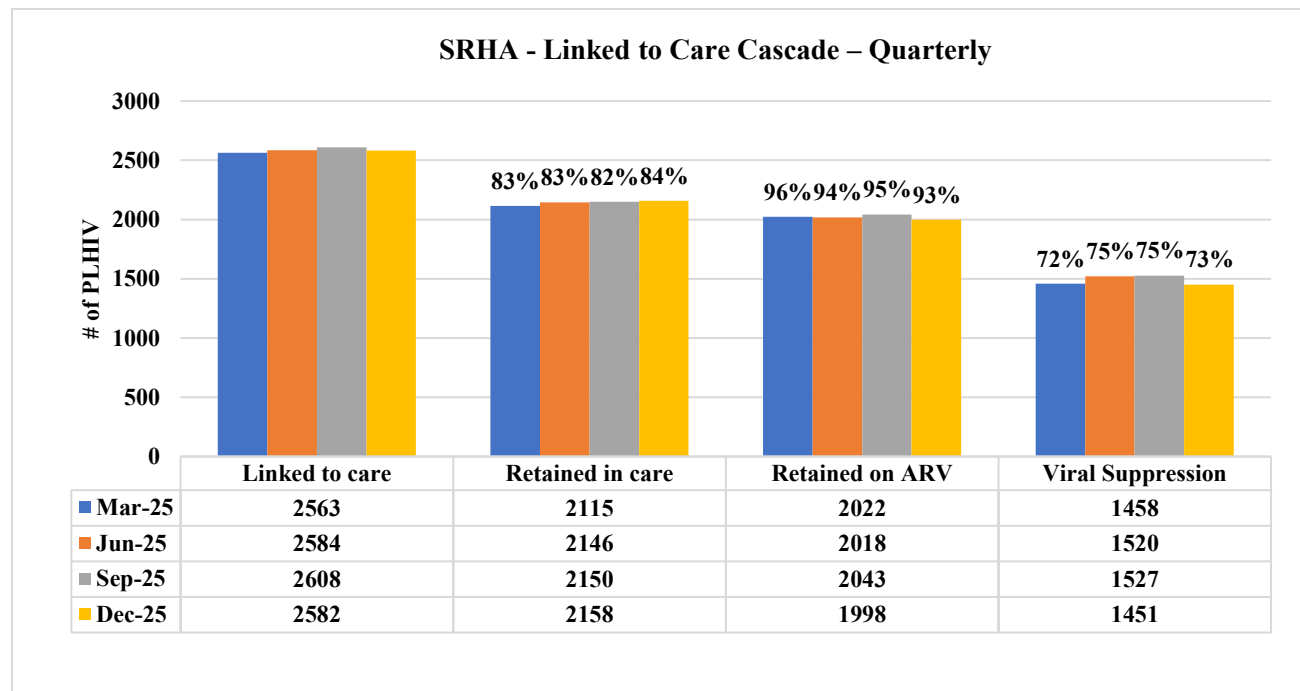


Figure 5: SRHA Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, SRHA's number of clients linked to care increased by 19. Among those linked to care, retention in care increased by 1%, from 83% in March 2025, when compared to 84% in December 2025.

Among those retained in care and on ARVs, the percentage declined by 3%, from 96% in March 2025, when compared to 93% in December 2025. This decline can be attributed to data entry error, missed appointments due to Hurricane Melissa and the timeliness of data entry. The percentage of PLHIV who were virally suppressed increased by 1%, from 72% in March 2025, when compared to 73% in December 2025, which can be attributed to increased viral load testing and more timely processing and entry of results into the database (Figure 5).

Western Regional Health Authority (WRHA)

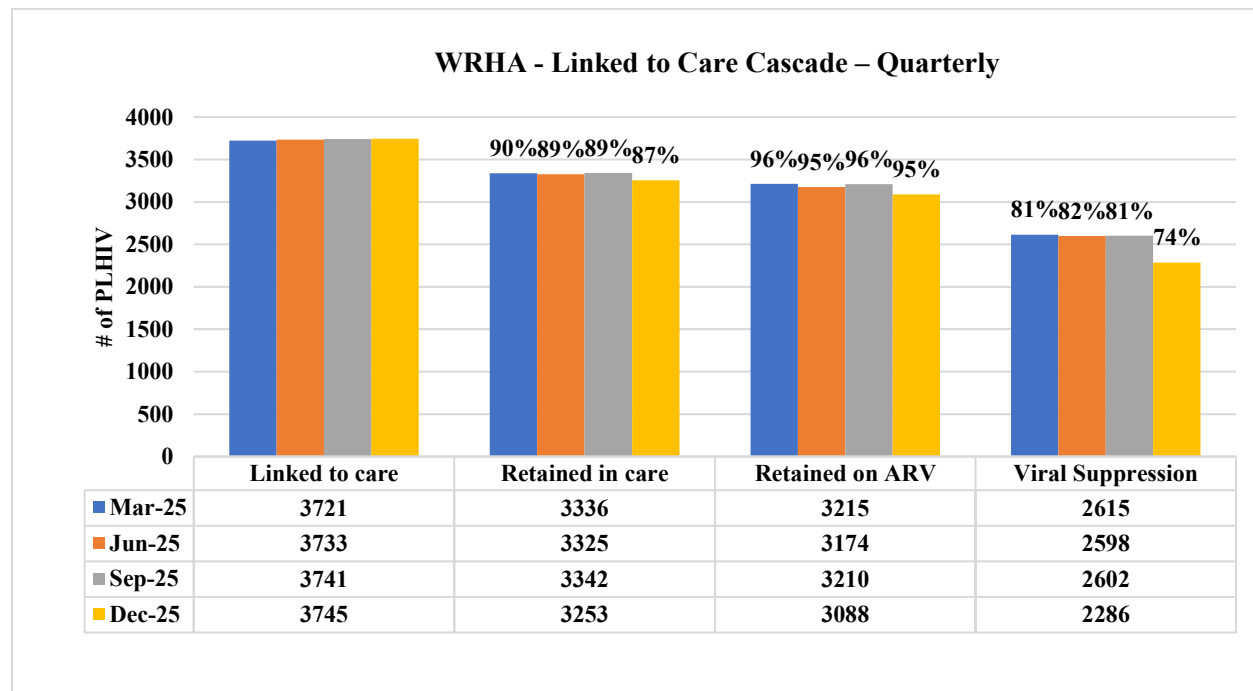


Figure 6: WRHA Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, WRHA's number of clients linked to care increased by 24. Among those linked to care, retention in care declined by 3%, from 90% in March 2025, when compared to 87% in December 2025. Among those retained in care and on ARVs, the percentage declined by 1%, from 96% in March 2025, when compared to 95% in December 2025. This decline can be attributed to data entry error, missed appointments due to Hurricane Melissa and delay in data entry.

The percentage of PLHIV who were virally suppressed declined by 7%, from 81% in March 2025, when compared to 74% in December 2025, which can be attributed to Hurricane Melissa, the delay of data entry, the delay in receiving viral load test results and patients refusing to get their viral load test done (Figure 6).

Non-RHA Organizations Linked to Care Cascade

The Non-RHAs 90-90-90 cascades assess the percentage achievement of the UNAIDS target to achieve epidemic control of HIV. This analysis helps determine how service providers manage the patients linked to care, retained on ARVs, and virally suppressed. The gaps at each pillar of the cascade were identified and can be used to plan targeted interventions to address the fallout at each pillar.

Jamaica AIDS Support for Life (JASL)

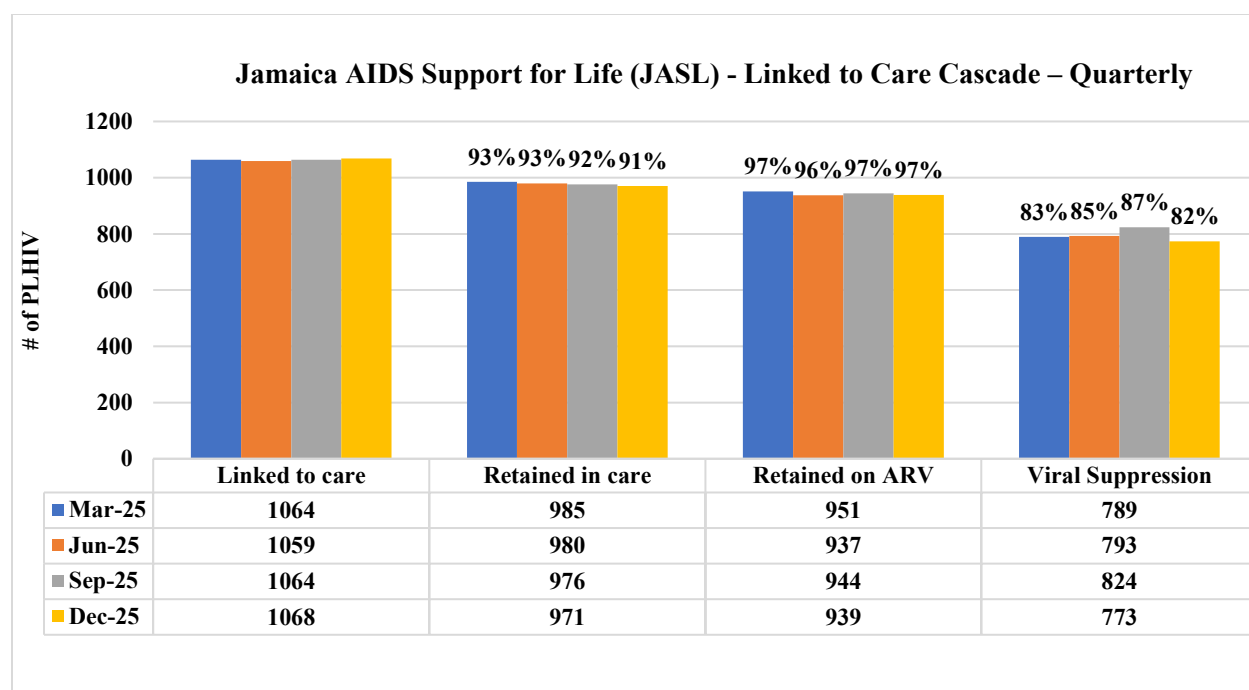


Figure 7: Jamaica AIDS Support for Life (JASL) Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, JASL's number of clients linked to care increased by 4. Among those linked to care, retention in care declined by 2%, from 93% in March 2025, when compared to 91% in December 2025.

Among those retained in care and on ARVs, the percentage remained the same at 97% in March 2025, when compared to December 2025. The percentage of PLHIV who were virally suppressed declined by 1%, from 83% in March 2025, when compared to 82% in December 2025. Of note is that viral suppression steadily increased between March to September 2025 (Figure 7).

CHARES

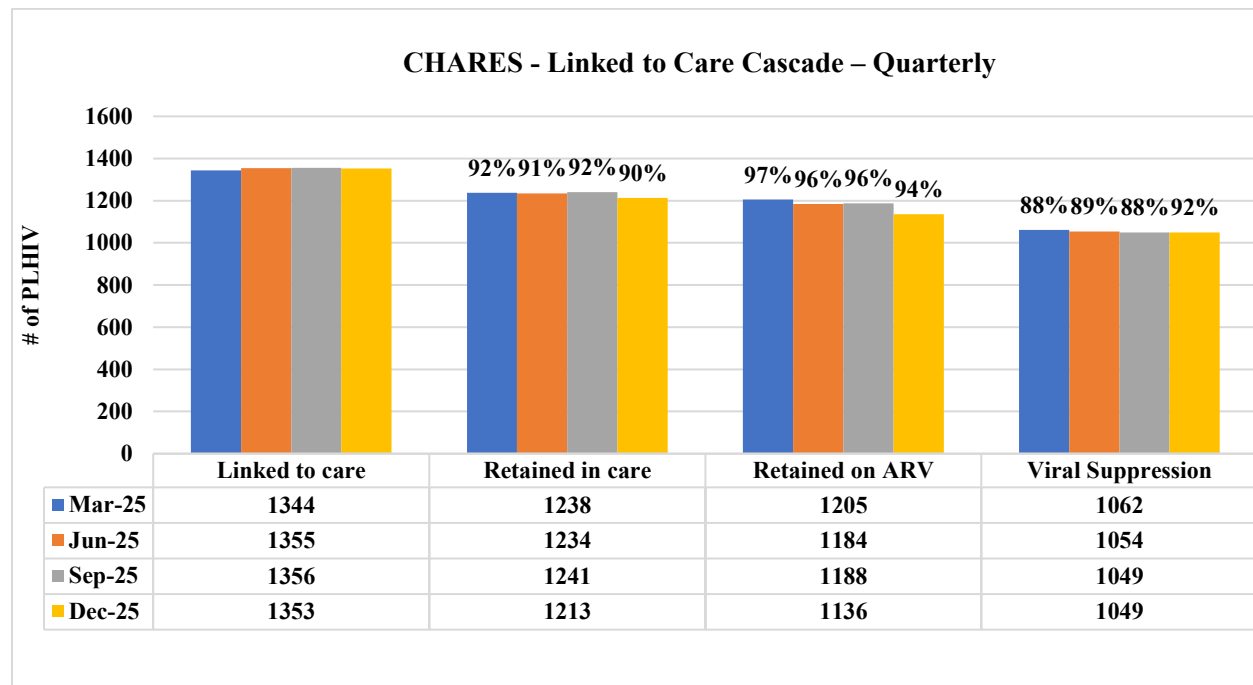


Figure 8: CHARES Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, CHARES' number of clients linked to care increased by 9. Among those linked to care, retention in care declined by 2%, from 92% in March 2025, when compared to 90% in December 2025.

Among those retained in care and on ARVs, the percentage declined by 3%, from 97% in March 2025, when compared to 94% in December 2025. The percentage of PLHIV who were virally suppressed increased by 4% from 88% in March 2025, when compared to 92% in December 2025 (Figure 8).

AIDS Healthcare Foundation (AHF)

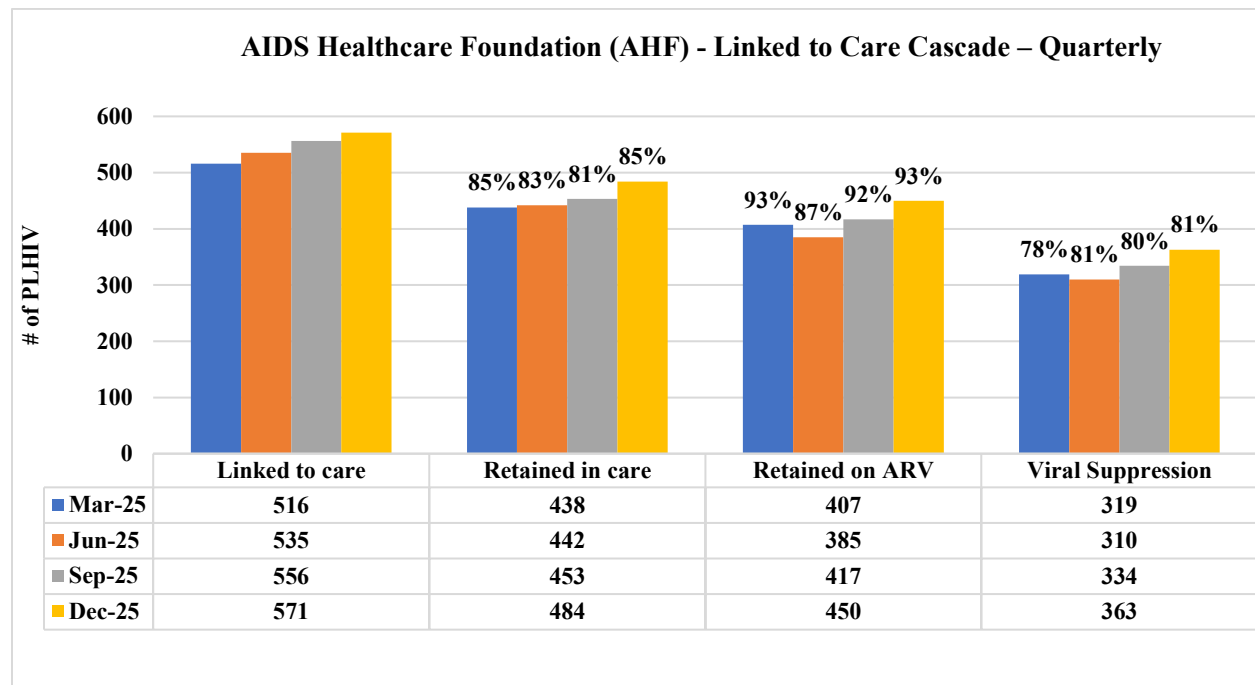


Figure 9: AIDS Healthcare Foundation (AHF) Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, AHF's number of clients linked to care increased by 55. Among those linked to care, retention in care remained the same at 85% in March 2025, when compared to December 2025.

Among those retained in care and on ARVs, the percentage remained the same at 93% in March 2025, when compared to December 2025. The percentage of PLHIV who were virally suppressed increased by 3%, from 78% in March 2025, when compared to 81% in December 2025 (Figure 9).

Health Connect Jamaica

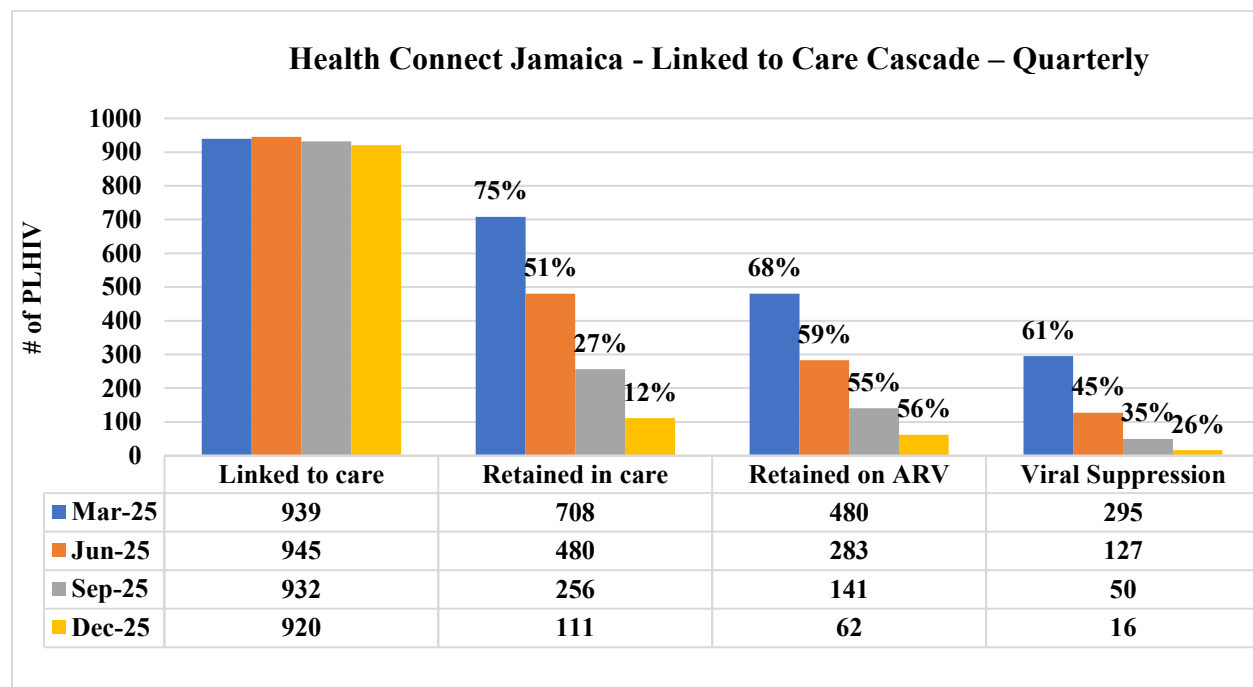


Figure 10: Health Connect Jamaica Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, HCJ's number of clients linked to care declined by 19. Among those linked to care, retention in care declined by 63%, from 75% in March 2025, when compared to 12% in December 2025.

Among those retained in care and on ARVs, the percentage declined by 12%, from 68% in March 2025, when compared to 56% in December 2025. The percentage of PLHIV who were virally suppressed declined by 35%, from 61% in March 2025, when compared to 26% in December 2025 (Figure 10).

There was a disruption in services offered at Health Connect Jamaica since January 2025 to present due to the freeze in support provided by the Government of the United States of America. It is highly likely that the decrease in performance noted on the cascade pillars is attributed to this disruption. A list of HCJ's patients will be provided to the regional contact investigators to re-engage them back into care.

UHWI – Paediatric Clinic

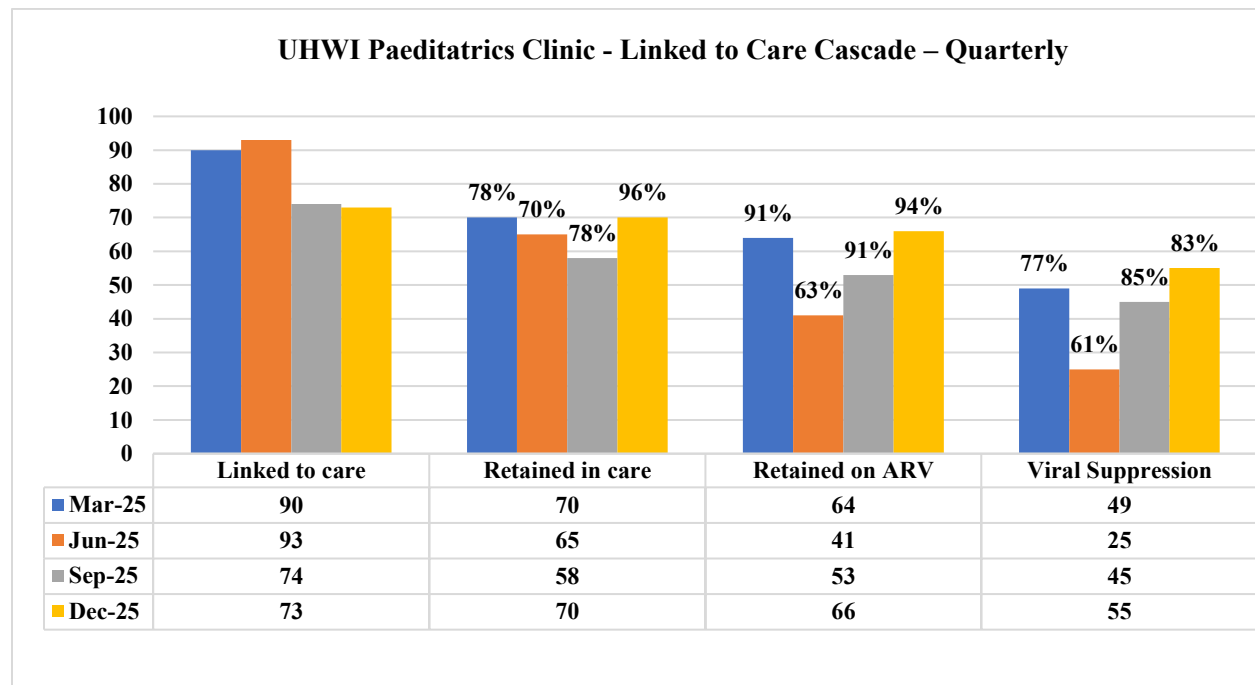


Figure 11: UHWI – Paediatric Clinic Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, UHWI- Paediatric Clinic number of clients linked to care declined by 17. This decline is attributed to data cleaning. Among those linked to care, retention in care increased by 18%, from 78% in March 2025, when compared to 96% in December 2025.

Among those retained in care and on ARVs, the percentage increased by 3%, from 91% in March 2025, when compared to 94% in December 2025. The percentage of PLHIV who were virally suppressed increased by 6%, from 77% in March 2025, when compared to 83% in December 2025 (Figure 11).

Bellevue Hospital

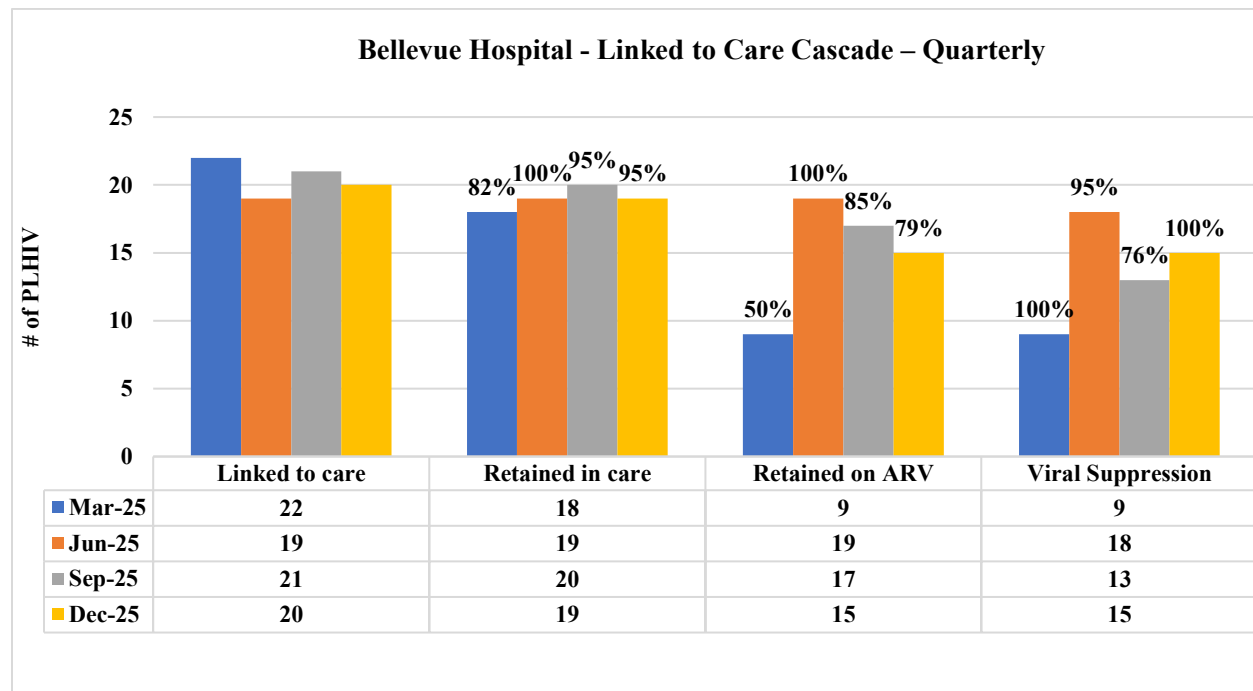


Figure 12: Bellevue Hospital Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, Bellevue’s number of clients linked to care declined by 2. Among those linked to care, retention in care increased by 13%, from 82% in March 2025, when compared to 95% in December 2025.

Among those retained in care and on ARVs, the percentage increased by 29%, from 50% in March 2025, when compared to 79% in December 2025. However, it is important to highlight that throughout the four quarters the percentages fluctuated drastically, this is due to a high staff turn around and the access to the computer for data entry is limited, resulting in delay in data entry. The site is requesting a designated computer which should clear the backlog and maintain consistent data entry. The percentage of PLHIV who were virally suppressed remained the same at 100% in March 2025, when compared to December 2025 (Figure 12).

Department of Correctional Facilities

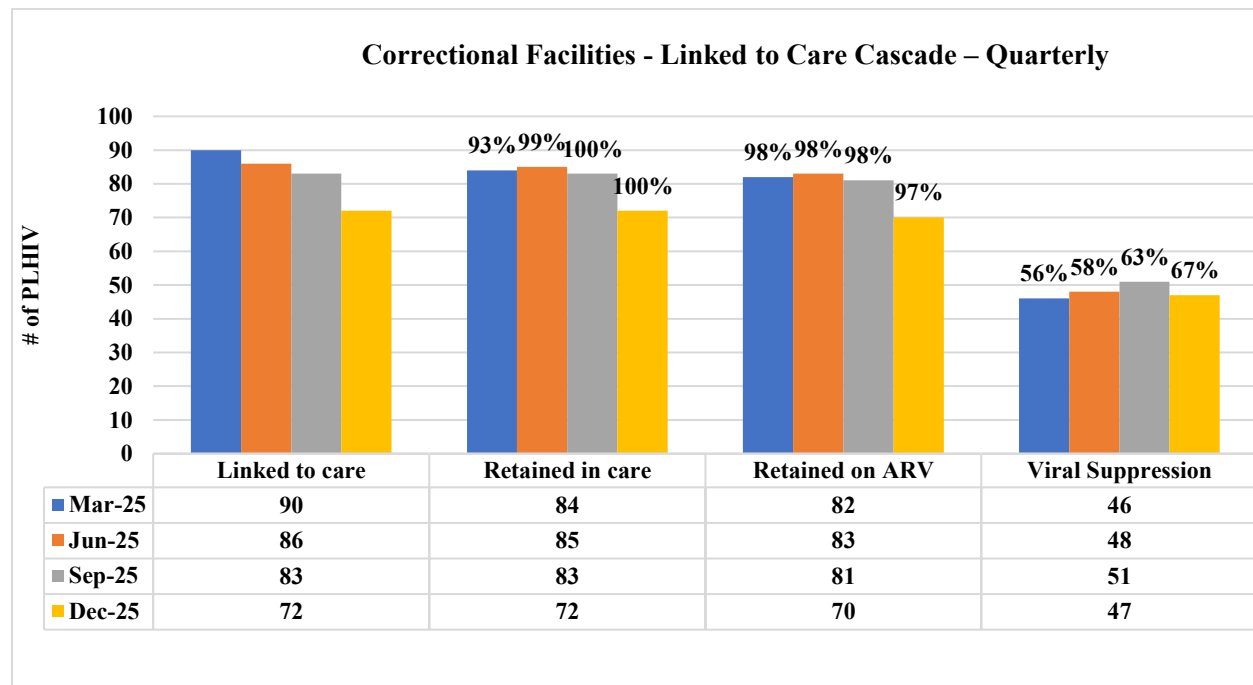


Figure 13: Department of Correctional Facilities Linked to Care Cascade – Quarterly

Comparing data from March 2025 to December 2025, the Department of Correctional Facilities' (DCS) number of clients linked to care declined by 18, due to data cleaning. Among those linked to care, retention in care increased by 7%, from 93% in March 2025, when compared to 100% in December 2025.

Among those retained in care and on ARVs, the percentage declined by 1%, from 98% in March 2025, when compared to 97% in December 2025. The percentage of PLHIV who were virally suppressed increased by 11%, from 56% in March 2025, when compared to 67% in December 2025 (Figure 13).

RHA Service Level Agreement Targets and Performance

Table 1: PLHIV Retained on ARV as of December 2025 matched against the Annual Target

Region	Parish	PLHIV Retained on ARV	On ARV 2025 Target	Percentage achieved
SERHA	St. Thomas	302	350	86%
	Kingston and St. Andrew	3,121	3,701	84%
	St. Catherine	1,846	2,054	90%
WRHA	Trelawny	358	387	93%
	Hanover	216	231	94%
	Westmoreland	675	837	81%
	St. James	1,839	2,102	87%
NERHA	Portland	308	362	85%
	St. Mary	466	542	86%
	St. Ann	847	897	94%
SRHA	Clarendon	836	943	89%
	Manchester	835	939	89%
	St. Elizabeth	327	437	75%
Total		11,976	13,782	87%

The total number of patients on ARV across the parishes as of December 2025 was 11,976. As of December 2025, no parish has met the On ARV target. However, it is important to note that St. Catherine, Trelawny, Hanover, and St. Ann have achieved a percentage of 90 or more thus far (Table 1).

Table 2: PLHIV Viral Load Testing of those eligible for a viral load test as of December 2025 matched against the Annual Target

Region	Parish	Viral Load Testing Achieved	Viral Load Testing Target	Percentage Achieved
SERHA	St. Thomas	281	90%	96%
	Kingston and St. Andrew	2,452		81%
	St. Catherine	1,603		90%
WRHA	Trelawny	305		88%
	Hanover	192		95%
	Westmoreland	494		76%
	St. James	1,460		81%
NERHA	Portland	281		94%
	St. Mary	426		93%
	St. Ann	696		85%
SRHA	Clarendon	662		82%
	Manchester	661		82%
	St. Elizabeth	242		77%
Total		9,755		84%

As of December 2025, St. Thomas, St. Catherine, Hanover, Portland, and St. Mary have met or surpassed the viral load testing target of 90%. The viral load testing being below the target at 84% was attributed to samples being compromised, viral load results not being entered correctly in TSIS, and patients not wanting to do their viral load test. The data quality issue was further investigated, and a meeting was scheduled with the TCSOs to enter missing viral load data and investigate those without a viral load test done. The outcome of this plan of action will be reviewed in the next TSIS Data Quality scheduled for January 2026 (Table 2).

Table 3: PLHIV Viral Suppression of those eligible for a viral load test as of December 2025 matched against the Annual Target

Region	Parish	Viral Suppression Achieved	Viral Suppression Target	Percentage achieved
SERHA	St. Thomas	268	90%	95%
	Kingston and St. Andrew	2,265		92%
	St. Catherine	1,403		88%
WRHA	Trelawny	278		91%
	Hanover	179		93%
	Westmoreland	447		90%
	St. James	1,369		94%
NERHA	Portland	262		93%
	St. Mary	389		91%
	St. Ann	637		92%
SRHA	Clarendon	604		91%
	Manchester	626		95%
	St. Elizabeth	215		89%
Total		8,942		92%

As of December 2025, the majority of the parishes met or surpassed the viral suppression target of 90%. However, St. Catherine and St. Elizabeth have room for improvement (Table 3).

Addressing the Gaps

The HIV Treatment Cascade (Figure 13) highlights critical stages in the continuum of care: diagnosis, linkage to care, retention in care, retention on antiretroviral therapy (ART), viral load testing and viral suppression. At each step, gaps emerge that weaken the overall effectiveness of the HIV response. One of the most significant gaps occurs after treatment initiation, where individuals are lost to follow-up, not retained on ART, and not virally suppressed. These disruptions can lead to increased morbidity, the development of drug resistance, and ongoing transmission within communities. Addressing these gaps, especially around retention and re-engagement, is essential to improving treatment outcomes and achieving epidemic control.

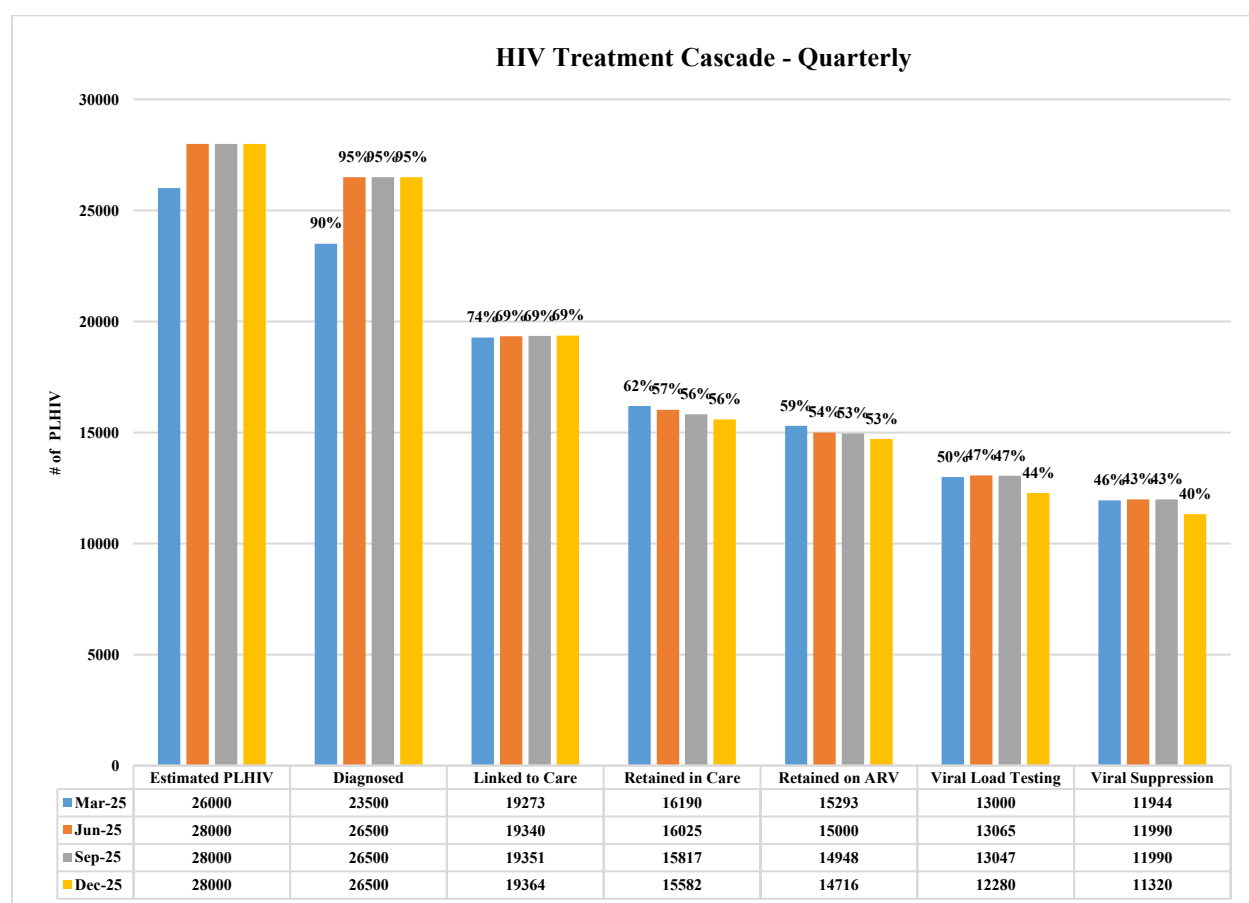


Figure 14: HIV Treatment Cascade - Quarterly

Figure 14 highlights the gaps between each pillar. Between the diagnosed and the linked to care pillars, the data shows that 7,136 PLHIV are not linked to care as of December 2025. Between the linked to care and retained in care pillars, the data shows that 3,782 PLHIV are lost to follow up as of December 2025. Between the retained in care and retained on ARV pillars, the data shows that 866 PLHIV are not retained on ARV. Between the retained on ARV and the viral load testing pillars, the data shows

that 2,436 PLHIV are without a valid viral load test result. Between the viral load testing and viral suppression pillars, the data show that 960 PLHIV are not virally suppressed.

Lost to Follow Up

The "Lost to Follow-Up" (LTFU) gap in the HIV cascade refers to individuals who were previously diagnosed and linked to care but have since disengaged from care for greater than 90 days since their last expected appointment date. This gap undermines the effectiveness of HIV programmes, as treatment interruptions can lead to poor health outcomes, increased viral load, and a higher risk of HIV transmission. Addressing LTFU requires a focus on patient tracking, reducing barriers to care, and implementing strategies to support long-term retention and re-engagement in treatment.

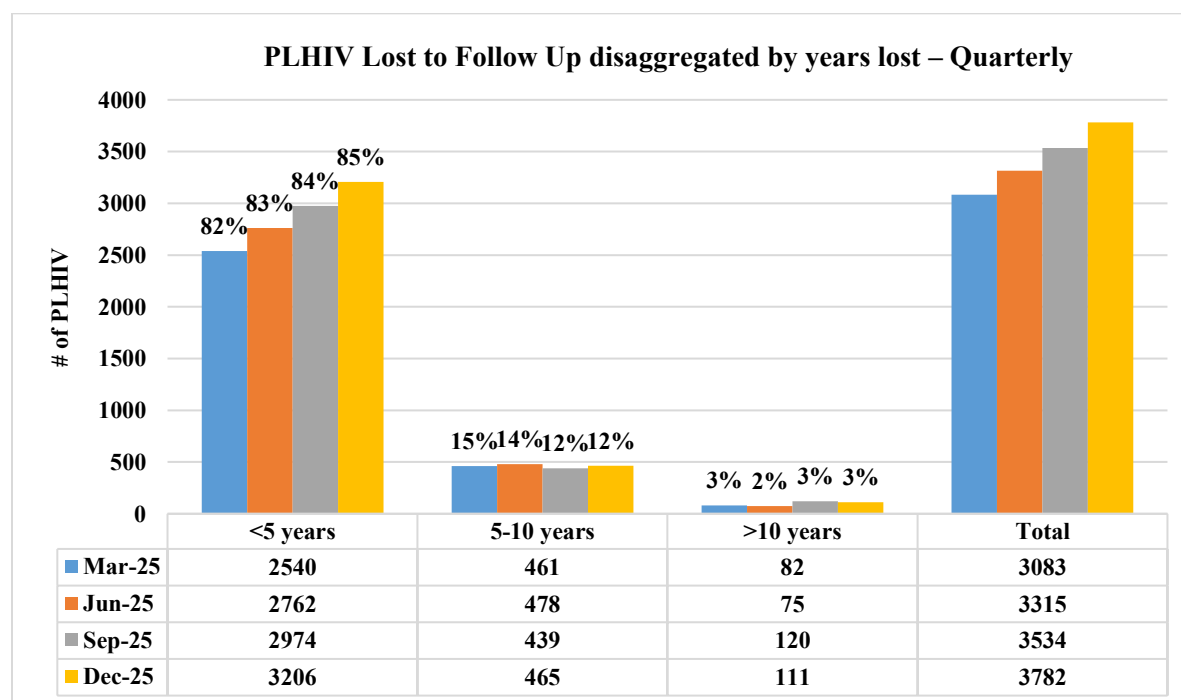


Figure 15: PLHIV Lost to Follow Up disaggregated by years lost – Quarterly

PLHIV lost to follow up increased by 666 from March 2025 to December 2025. The number of PLHIV lost to follow up for less than 5 years is highest compared to those lost for 5-10 years and greater than 10 years. PLHIV lost to follow up for less than 5 years increased by 3% from March 2025 to December 2025 at 85%. PLHIV lost to follow up for 5 – 10 years declined by 3% from March 2025 to December

2025. PLHIV lost to follow up for greater than 10 years remained at 3% for March 2025 to December 2025 (Figure 15).

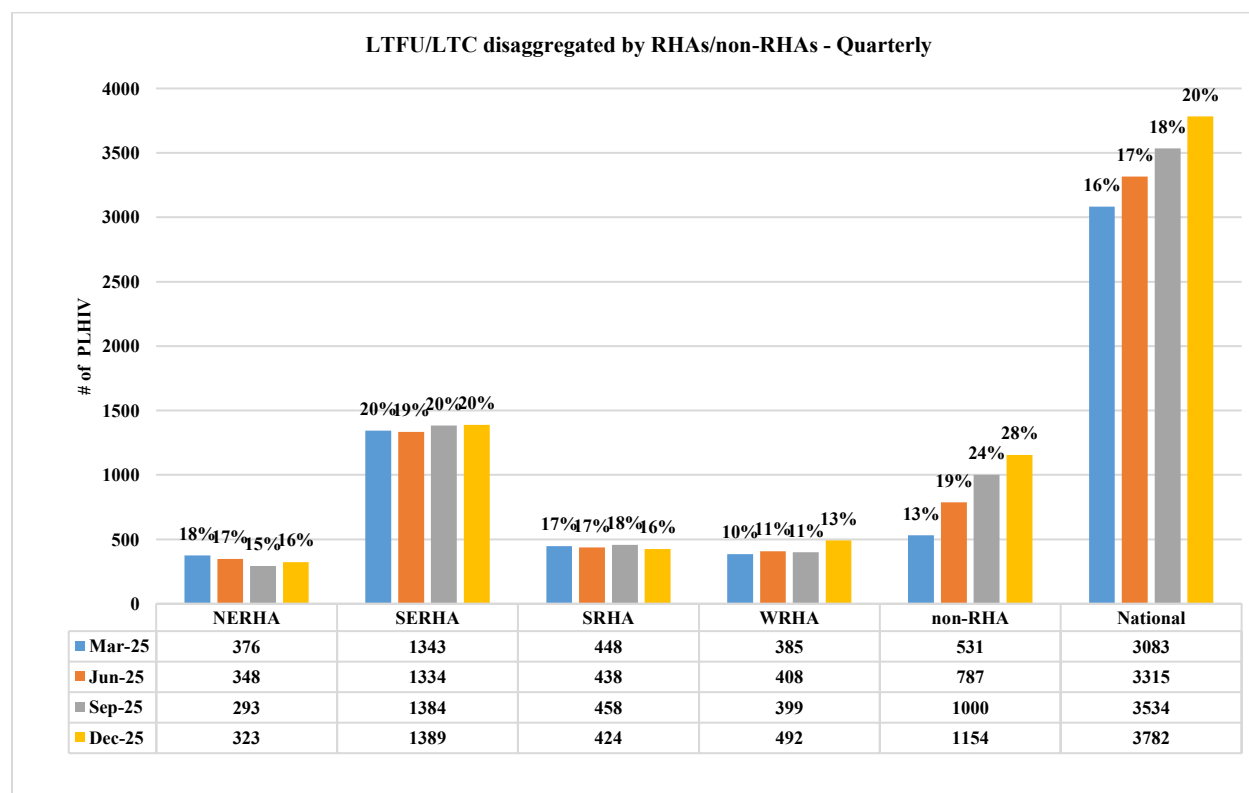


Figure 16: LTFU/LTC disaggregated by RHAs/non-RHAs – Quarterly

Nationally, a 4% increase was seen in lost to follow up of PLHIV linked to care from March 2025, when compared to December 2025. Lost to follow-up as of December 2025 were highest among the non-RHAs at 1,154, 28% of their linked to care, a drastic increase by 15% seen from March 2025, when compared to December 2025. SERHA's lost to follow up was found to be 20% of their linked to care, the same as March 2025, when compared to December 2025. SRHA's lost to follow up was found to decline by 1%, 17% from March 2025, when compared to 16% December 2025. NERHA's lost to follow up was found to be 16% of their linked to care, a 2% decline from March 2025, when compared to December 2025. WRHA's lost to follow up was found to be 13%, which increased by 3% from March 2025, when compared to December 2025 (Figure 16).

PLHIV Not Retained on ARV

Within the HIV treatment cascade, a key gap exists among individuals who are engaged in care but are not retained on antiretroviral therapy (ART). This group includes clients who are documented as being on treatment but are in fact antiretroviral (ARV) naïve, as well as those affected by data entry issues. One such issue is the entry of prescription end dates that expire before the client's next scheduled appointment, leading to an appearance of treatment interruption in the database. Although these individuals may be accessing services, their inconsistent treatment status poses risks to their health and undermines viral suppression efforts. Strengthening data quality and ensuring continuity of treatment while in care are critical to closing this gap.

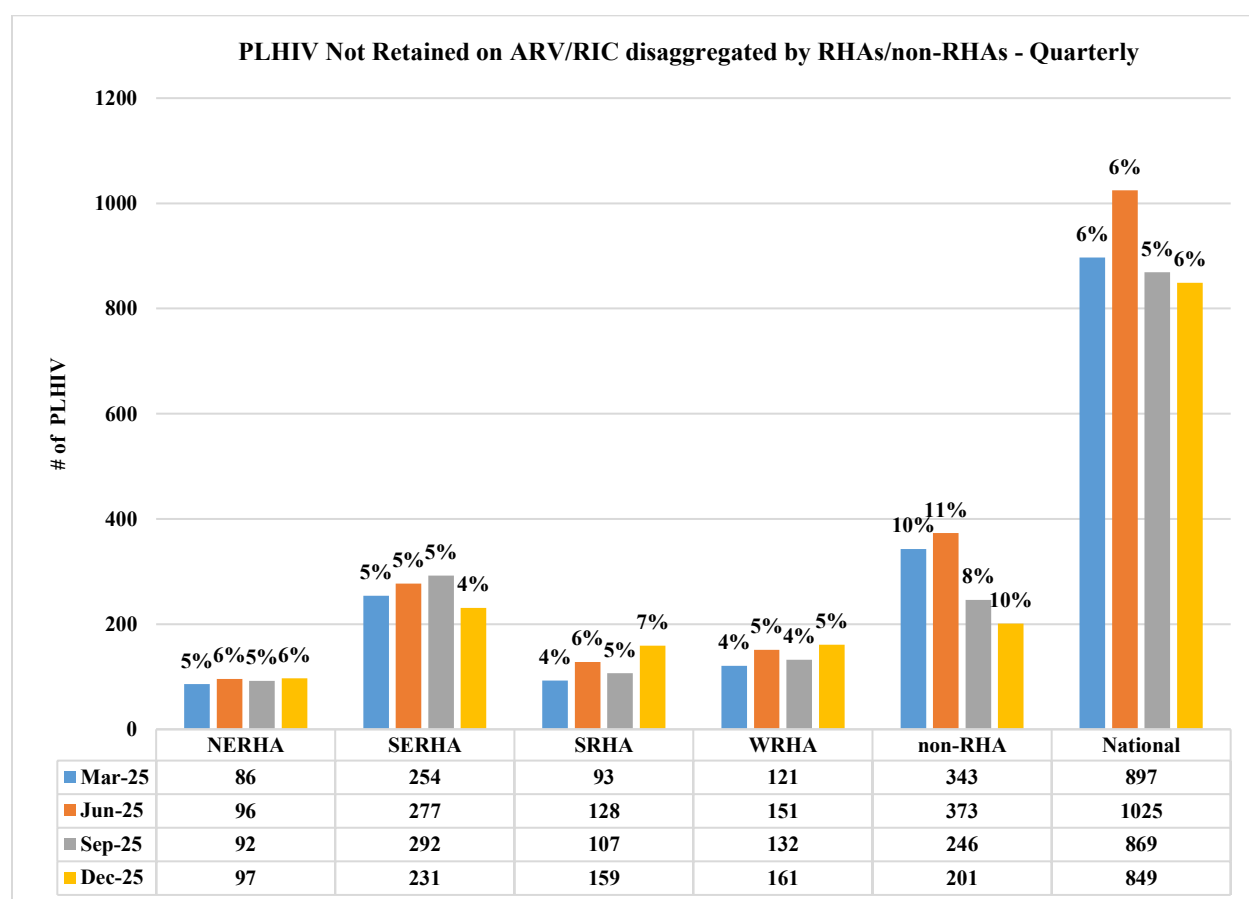


Figure 17: PLHIV Not Retained on ARV/RIC disaggregated by RHAs/non-RHAs - Quarterly

Nationally, PLHIV not retained on ARV remained the same from March 2025, when compared to December 2025. SERHA's PLHIV not retained on ARV declined by 1% from 5% in March 2025, to 4% in December 2025. SRHA's PLHIV not retained on ARV was found to be 7% of their retained in care, an increase by 3% from March 2025, when compared to December 2025. NERHA's PLHIV not retained on ARV was found to be 6% of their retained in care, a 1% increase from March 2025, when compared to December 2025. WRHA's PLHIV not retained on ARV was found to be 5% of their retained in care, a 1% increase from March 2025, when compared to December 2025. The non-RHAs

PLHIV not retained on ARV was found to be 10% of their retained in care, remaining the same from March 2025, when compared to December 2025 (Figure 17).

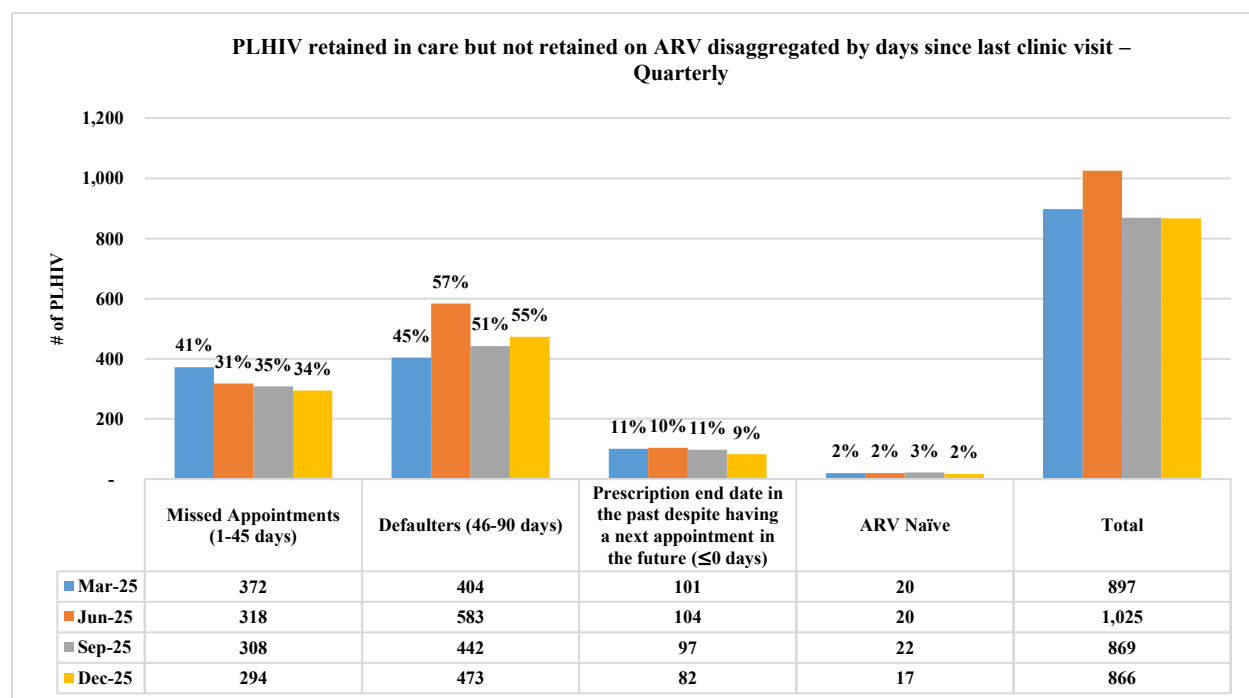


Figure 18: PLHIV retained in care but not retained on ARV disaggregated by days since last clinic visit – Quarterly

PLHIV retained in care but not retained on ARV disaggregated by days since last expected clinic visit found that missed appointments (1-45 days) declined by 7% from March 2025 to December 2025. Defaulters (46-90 days) increased by 10% from March 2025 to December 2025. The incorrect prescription end date declined by 2% from March 2025 to December 2025. The number of PLHIV that are ARV naïve remained the same at 2% from March 2025 to December 2025 (Figure 18).

PLHIV with No Valid Viral Load Test Result

Another critical gap in the HIV treatment cascade is the absence of a valid viral load (VL) test result for individuals on antiretroviral therapy (ART). This gap may result from missed appointments, delayed sample collection, rejected samples, or outdated test results that fall outside the reporting period. In some cases, clients remain in care and on treatment, but without a recent or valid VL result, their treatment response cannot be accurately monitored. This limits the ability to assess programme performance and may delay necessary clinical interventions. Addressing this gap requires strengthened sample management, timely result entry, and improved coordination between clinical and laboratory teams to ensure that all clients on ART have up-to-date and valid viral load results.

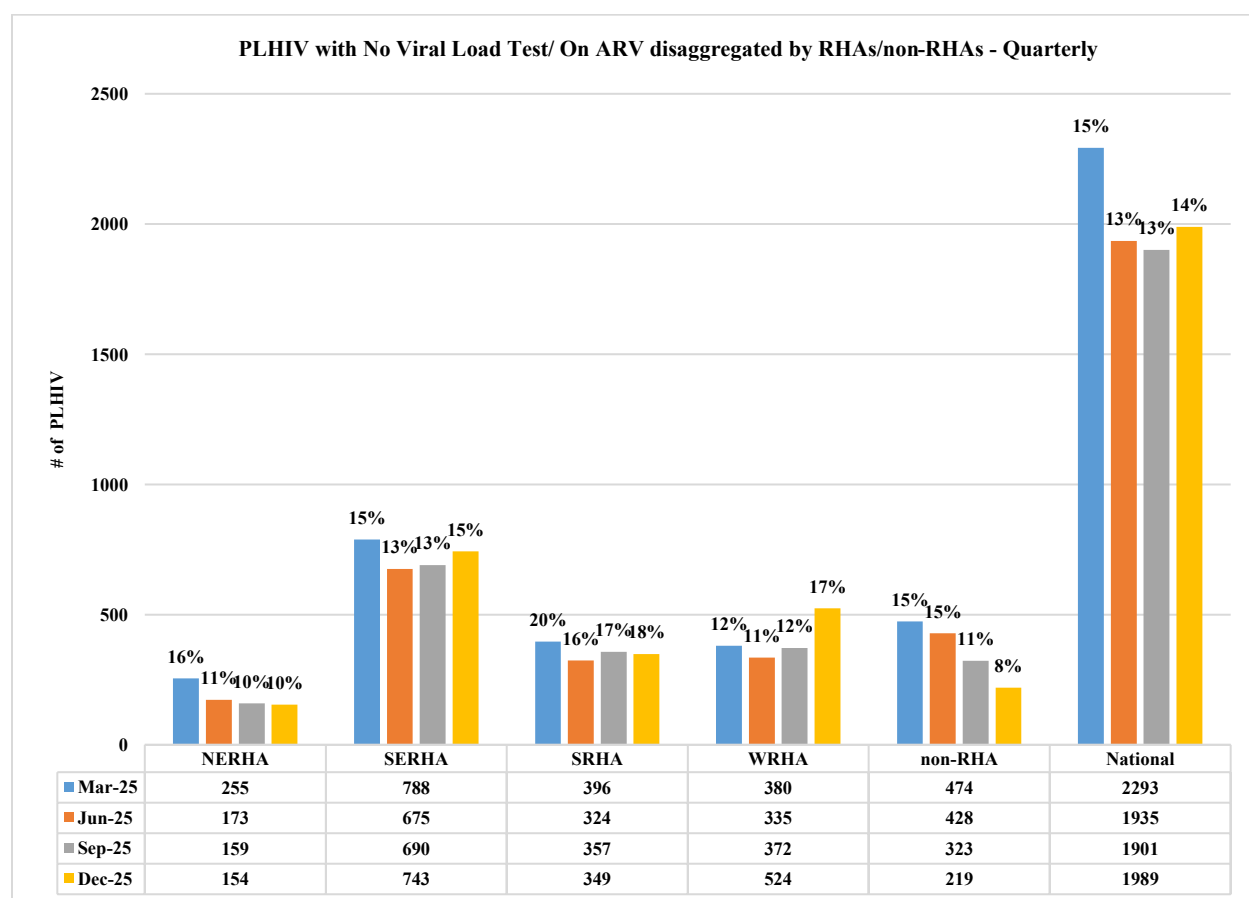


Figure 19: PLHIV with No Viral Load Test/ On ARV disaggregated by RHAs/non-RHAs - Quarterly

Nationally, a 1% decline was seen in PLHIV not retained on ARV from March 2025, when compared to December 2025. PLHIV with no valid viral load result as of December 2025 were highest among the SRHA at 18% of their retained on ARV, with a 2% decline seen from March 2025, when compared to December 2025. NERHA's PLHIV with no valid viral load result was found to be 10% of their retained on ARV, with a 6% decline seen from March 2025, when compared to December 2025. SERHA's PLHIV with no valid viral load result was found to be 15% of their retained on ARV, remaining the same from March 2025, when compared to December 2025. WRHA's PLHIV with no valid viral load result was found to be 17% of those retained on ARV, with a 5% increase from March

2025, when compared to December 2025. Non-RHA's PLHIV with no valid viral load result were found to be 8% of those retained on ARV, with a 7% decline seen from March 2025, when compared to December 2025 (Figure 19).

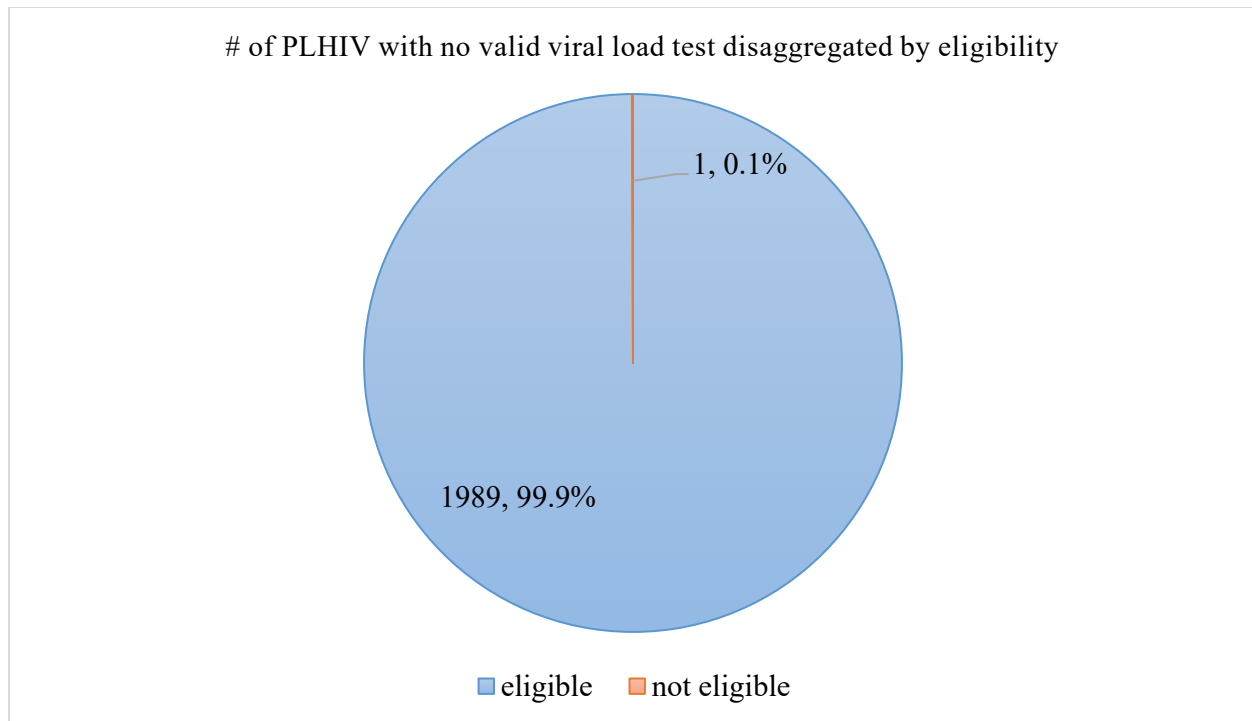


Figure 20: Number of PLHIV with no valid viral load test disaggregated by eligibility

Majority (99.9%) of the PLHIV with no valid viral load test results are eligible for a viral load test based on the HIV treatment protocol which states PLHIV are eligible for a viral load test: six months after initiation on ARV, six months after last unsuppressed viral load, six months after first suppressed viral load and twelve months for suppressed clients with two consecutive valid suppressed viral load results (Figure 20).

Plan of Action to Address Data Quality Gaps

During supportive supervision visits the focal points, for each region and non-RHAs, from both SI and TCS will use the actions listed below to address the gaps identified in this report. In addition, monthly meetings hosted by the Strategic Information Component with the Treatment, Care and Support Officers will be used to address the issues identified.

Issues Identified	Action Item	Responsible Officer
Incorrect ARV Prescription End Dates	- Conduct routine audits of ARV prescription end dates to identify treatment interruptions. - Introduce TSIS system flags for expired prescriptions and missed refills.	- Strategic Information Component (SI) and the Treatment, Care and Support Officers (TCSOs) - SI
Missing or Invalid Viral Load Results	- Monitor for VL test requests that are missing, outdated, or outside the reporting period. - Ensure all VL results are entered promptly and accurately in TSIS.	- TCSOs - TCSOs
Incomplete or Inconsistent Data Entry	- Provide refresher training for data clerks and clinicians on key data fields (ART, VL, appointments). - Implement validation rules in TSIS to flag common entry errors (e.g., expired prescriptions, missing VL).	- SI - SI
General Data Oversight	- Establish routine site-level data quality checks (monthly or quarterly). - Generate and review exception reports to proactively identify and address anomalies in real-time.	- SI and TCSOs - SI
Unrecorded Transfers or Deaths	- Collaborate with facilities to verify silent transfers and remove them from LTFU lists. - Update records for deceased clients.	- SI - TCSOs
Delayed Data Entry	- Minimize lag between client visits and data entry by improving real-time or end-of-day data updates.	- TCSOs

Plan of Action to Address Programmatic Gaps

Main Challenge	Plan of Action
Not Retained on ART	● Develop site specific retention improvement plans. ● Implement LTFU –RTC Activities. ● Extended clinic hours, increased days for service delivery (weekend clinic) and increase in health facilities able to offer HIV prevention, treatment and care. ● Strengthen client tracking and follow-up mechanisms. ● Expand community-based ART delivery and multi-month dispensing. ● Provide transportation support and patient education to address stigma and barriers.
Low Viral Load Uptake	● Task shifting of staff to provide venipuncture and sample collection ● Mop up of outstanding viral load samples during the extended hours clinic ● Decentralizing Viral load testing ● Expand mobile phlebotomy and outreach services in rural areas. ● Strengthen sample transport logistics and tracking.
Low Viral Suppression	● Case Management ● Differentiated Services of Care ● HIVDR services

Conclusion

While progress has been made in HIV care and treatment, gaps such as clients not being retained on ART and the absence of valid viral load test results highlight areas where continued attention is needed. In many cases, individuals remain engaged in care, but inconsistencies in treatment and monitoring limit the ability to fully assess and support their outcomes. These gaps are often influenced by data quality issues, system inefficiencies, and individual-level challenges. Strengthening data management, improving retention efforts, and enhancing coordination between clinical and laboratory services will support more consistent care and better health outcomes. Addressing these areas will help to optimize the treatment cascade and contribute to national goals for HIV control and long-term client well-being.